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June 16, 2025

Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
7500 Security Boulevard
Baltimore, MD 21244-1850

RE: Request for Information on the Health Technology Ecosystem (CMS-0042-NC) Submitted Electronically via Regulations.gov

Dear Deputy Administrator Carlton and Acting Assistant Secretary and Acting National Coordinator Posnack,

On behalf of the American Urological Association (AUA) and our more than 23,000 members, we appreciate the opportunity to respond to the Centers for Medicare & Medicaid Services (CMS) and the Office of the National Coordinator for Health IT (ONC) Request for Information on the Health Technology Ecosystem. We commend CMS and ONC for seeking broad stakeholder input and for recognizing that the path to digital transformation must center both provider and patient realities. Our comments are below.

About AUA

The AUA is a globally engaged organization with more than 23,000 physicians, physician assistants, and advanced practice nursing members practicing in more than 100 countries. Our members represent the world's largest collection of expertise and insight into the treatment of urologic disease. Of the total AUA membership, more than 13,000 are based in the United States and provide invaluable support to the urologic community by fostering the highest standards of urologic care through education, research, and the formulation of health policy.

1. Data Exchange [PA-2, PA-5; PR-3]

We urge CMS to lead on making clinical data exchange real-time, reliable, and built into workflows. That starts with giving specialty registries the infrastructure support they need. Registries like the AUA's Quality AQUA Registry are powerful tools, but only if they are integrated into how we practice. CMS should push for application programming interface (APIs) and automation that align registry reporting with how clinicians already use

electronic health records, instead of asking us to duplicate work **[PA-2].**

In parallel, CMS should work with vendors to develop smarter, dual-purpose documentation tools. What providers document for patient care should also satisfy reporting requirements. There is no reason providers should be entering the same information into different systems to meet different metrics. Creating a single submission platform that works across CMS and commercial payers would be particularly impactful for small practices that do not have dedicated administrative teams **[PA-5]**.

Beyond registries and reporting, CMS must also confront the reality that meaningful data exchange is still hindered by barriers to full chart access **[PR-3]**. Despite widespread use of patient portals, both patients and physicians often struggle to retrieve comprehensive clinical information. Portals frequently offer only fragments of the record, such as lab values without notes, summaries without context, or scanned documents withheld entirely. Even providers using the same electronic health record (EHR) system but operating in different practices or health systems cannot always access a complete picture. That gap undermines care coordination, wastes resources, and delays treatment.

There is no good reason a treating physician should be blocked from accessing their patient's operative note or pathology report when that information exists elsewhere in the system. We hear repeatedly from patients and providers about the inefficiency of additional phone calls, forms, and faxes to obtain information that should be available immediately through the EHR. Full, real-time chart access across vendors and systems is foundational to safe, efficient, and accountable care.

Finally, any CMS endorsement of third-party networks or registries must be based on clear, public criteria that prioritize ease of use, interoperability, and vendor neutrality. Right now, the patchwork of options only makes life harder for clinicians, a burden on top of providing quality patient care.

2. Data Access and Integration [PC-2, PC-4, PC-10; PR-5]

The most important aspect of digital health is complete and timely access to data. Clinicians and patients alike need to see more than just lab results. Operative reports, pathology records, imaging summaries and other such documentation is critical to care planning, especially in fields like urologic oncology where treatment decisions are highly dependent on longitudinal data.

AUA received multiple comments from patients on [**PC-2]** and [**PC-4]**, all of which wished for a more-user friendly experience that combines data access with the ability to take action, either for themselves or for their loved ones. One patient wished for a "diagnostic tracker" that denotes changes across multiple health conditions simultaneously, allowing them to follow how those conditions are advancing, another asked for seamless calendar integration, where appointment automatically sync to the patient's or caregiver's calendar app. Another patient similarly asked for automatic calendar syncing between members of the care team, so that when an oncologist changes an appointment, for instance, the infusion office appointment is automatically updated too.

Multiple patients asked for the ability to direct message administrative members of the health care team and to submit forms electronically instead of having to mail or hand-deliver them.

Similarly, urologic patients asked for the ability to get more information about insurance decisions and explanations of benefits in one click, and to easily match explanations of benefits to bills. In short, patients want easy to use apps that can connect them directly with a person, and that intuitively match different data elements so that they can better understand care and coverage decisions.

Health information exchanges (HIEs) **[PC-10]** were supposed to solve some of these problems, but in practice, they have come up short. Too many are fragmented, expensive, or suffer from low participation. CMS should take a hard look at whether the Trusted Exchange Framework and Common Agreement or trusted vendor-based networks can do the job more effectively. Whatever framework is chosen, it needs to provide secure, bidirectional access that functions work in real-time and supports independent practices.

We also have concerns about bulk Fast Healthcare Interoperability Resources **[PR-5]** and similar tools being used to extract data from EHRs without giving clinicians meaningful access to the results. If CMS is going to pull clinical data for quality measurement, risk adjustment, or payment validation, then the providers generating that data, especially those in smaller practices without dedicated IT support, should also be able to view and use the insights produced. Having access to performance metrics, benchmarks, and care patterns would help clinicians identify opportunities for improvement, track outcomes over time, and better engage with value-based care programs. Clinicians need to be included in the feedback loop so that data collection leads to informed action, not just external evaluation.

3. Value-Based Care [VB-3]

If CMS wants broader adoption of value-based care, it needs to make participation feasible for more than just large health systems **[VB-3]**. Urology and other specialty practices are ready to engage in alternative payment models (APMs), but right now, the tools required are either too expensive, too siloed, or just not built with smaller practices in mind.

CMS should invest in modular, scalable information technology upgrades that allow practices to gradually build the infrastructure they need. That includes tools for population health tracking, predictive analysis, and actionable data dashboards. These tools are not optional enhancements; they are essential components for meaningful participation in value-based care models.

The digital health tools that would help us succeed, such as apps that streamline prior authorization, quality reporting, and real-time documentation, already exist but widespread adoption is blocked by a lack of integration standards and funding. CMS should support pilot programs that validate and deploy these specialty-focused tools in real clinical settings, with a focus on usability and return on investment for the provider.

4. Information Blocking [PR-12]

CMS and ONC must stop focusing solely on providers when analyzing information blocking issues. The real barriers often come from large health systems and EHR vendors who hide behind privacy arguments while protecting their market share **[PR-12]**. Independent physicians regularly encounter blockades when trying to get the data they need to treat patients safely and effectively.

As such, enforcement and disincentives should apply to all stakeholders, not just providers. CMS should also simplify the complaint process and revise the exception language so that it is actionable. Unfortunately, it is currently too complicated and open to interpretation, which only encourages obstruction.

Transparency and interoperability are essential for building trust in the system. When patients or referring physicians are denied access to complete records, it undermines care and wastes time and resources. Fortunately, CMS has the authority to change this policy and we urge you to do so.

Conclusion

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We applaud CMS for recognizing that the current digital health environment can burden the physicians delivering care and the patients receiving care. CMS has an opportunity to change the system by centering usability, transparency, and alignment by reducing redundancy, building tools that serve clinical care and reporting simultaneously, and giving smaller practices a fair shot at participating in value-based care.

We urge CMS to act swiftly and boldly on these issues. We stand ready to partner with you on these important solutions. Please do not hesitate to reach out to Maureen Cones, Esq., AUA's Chief Legal & Policy Officer, at mcones@AUAnet.org with any questions.

Sincerely,

Mark Edney, MD, MBA, FACS
Chair, Public Policy Council
American Urological Association